UTM UNIVERSITI TEKNOLOGI MALAYSIA

FACULTY OF COMPUTING
UTM Johor Bahru

SECP 3204: Software Engineering

Semester 01, 2026/2027

PROJECT PROPOSAL

AUTISM EARLY SCREENING SUPPORT SYSTEM

EXCELLENT SYNERGY RESOURCES

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1.0 Introduction

1.1 Need (N)

The current system for early detection of Autism Spectrum Disorders is really critical to allow timely and accurate decisions. However, a study shows that it appears to be that the existing screening processes are often fragmented and quite difficult for families to access, which makes this system have its weaknesses (Nasir et al., 2024) . This problem, which can have a big impact, also stems from the lack of specialists and facilities that are supposed to manage this process, leading to a backlog of patients waiting (Gordon-Lipkin et al., 2016) . The cost of care is also a challenge for families who are financially less capable. The existing screening process through facilities can lead to issues of unequal access. In fact, a large portion of the population gets less service for this reason. Therefore, considering the limitations of the traditional system, there is an urgent need for an alternative (non-traditional) screening method through the Autism Early Screening Support System. The system is needed to bridge this access gap by providing a screening platform that is more flexible and easily accessible to families from various socioeconomic backgrounds (Roux et al., 2012).

1.2 Approach (A)

By preventing the scattered screening process and difficult to access, this project suggested a development system which we called “Autism Early Screening Support System”. This system will be developed using a cross-platform approach that supports web-based platforms to ensure easy access for all target users, including parents, and non-governmental organizations (NGOs) as system owners. The system is completed by the core modules including Profile Management Module referring to the official standards of the Disabled Registration Application Form (BPPOKU Amendment 2019 - Section A). In addition, the Screening module is based on the M-CHAT-R clinical guidelines, equipped with an automatic scoring algorithm to effectively categorize risk levels into low, moderate, or high. Our system also supports notification, reporting and analytic data for system owner’s reference. The unique module of our system is behaviour monitoring.

1.3 Benefit (B)

Our system offers impactful benefits for the big three stakeholders which are Caregivers/Parents, Autistic Children and NGO. For Parents/Caregivers, this system enables them to conduct M-CHAT-R screenings independently from home that helps to solve the issue of delay in identifying potential autism risks. Manual paperwork can be reduced significantly by digitizing profile management directly aligned with the official **BPPOKU Pindaan 2019 (Bahagian A)** standards. For Autistic Children,this module continuously monitors, analyzes, and records children's emotional and behavioral patterns, helping caregivers identify the causes of their actions and offectively assess their emotional development. For NGO, the reporting module can

centralize the analytics dashboards which provide real-time reports and aggregated demographic data, assisting the NGO in evaluating screening trends and optimizing community outreach initiatives.

1.4 Competitor (C)

Most public health clinics rely heavily on physical paper forms for the M-CHAT-R questionnaire and manual hardcopy submissions for the *BPPOKU* registration. Unlike manual methods that are prone to delays, lost documents, and fragmented record-keeping, the **Autism Early Screening Support System** digitizes the entire workflow into a seamless cross-platform solution (web-based platform), featuring automated M-CHAT-R risk scoring algorithms and instant digital profile generation aligned with *BPPOKU Pindaan 2019* (Steinman et al., 2022).

2.0 Existing System

The existing systems related to early screening and detection of Autism Spectrum Disorder (ASD) in children includes standardized screening questionnaires and computer-based screening platforms. It helps parents, caregivers and healthcare professionals identify the early signs of ASD and determine whether further assessment is needed.

In Malaysia, quality of life for individuals with ASD can be improved by increasing awareness and access to services. For diagnosis and screening process, it started with routine screening at Klinik Kesihatan (KK) where the child will go through screening using Modified Checklist for Autism (M-CHAT-R). The screening process will be done during routine child health visits at age 18, 24, and 36 months. For comprehensive assessment, children will be referred to Family Medical Specialists (FMS), Pediatricians, or Child Psychiatrists for diagnosis if they were flagged during screening. In fact, therapy session can be accessed by the diagnosed children at KK, hospitals, community rehabilitation centers (PDK), or private therapy centers. Hence, it is crucial for parents to ensure timely intervention by seeking early screening at nearest KK (Vital Signs Editorial, 2025).

2.1 Modified Checklist for Autism in Toddlers, Revised with Follow-Up (M-CHAT-R)

M-CHAT-R is a standardized screening questionnaire which is designed to identify children who may be at risk of ASD. It can be completed by parents or caregivers to assess the early developmental behaviour of the children. If the initial results of the questionnaire indicates that the children are at potential risk, a follow-up question and further evaluation can be conducted to clarify the results. According to NUHS, a large-scale pilot study which conducted between August 2020 and November 2022 involves 5336 multi-ethnic children, who are aged between 17 to 20 months shows that the tool is effective for early autism detection in primary care settings (National University Health System, 2024).

2.2 Manual BPPOKU Registration Process

The manual BPPOKU registration process involves preparing and submitting the required information along with the supporting documents for disability registration. The eligibility requirements must be fulfilled first before continuing with the registration process. There are two ways to register BPPOKU which is manually, where the submission of the documents must be submitted at JKM counter and online via MyDaftar OKU (Government of Malaysia, 2026). Manual registration involves hardcopy forms where it is used to record and submit registration information. In fact, managing physical documents can make information retrieval and record management more difficult as screening results and registration information may be handled separately by the administrators. Hence, the proposed system aims to digitize the relevant workflow to generate a digital profile aligned with the BPPOKU *Pindaan 2019* requirements.

2.3 AutiScan: AI-Based Early Screening System for ASD

AutiScan is an AI-based web platform that is designed to support early autism screening of children who are aged between 1 to 4 years. The platform provides preliminary risk assessment by using a structured questionnaire with behavioural analysis and using uploaded video data. It also used machine learning models including Random Forest and Convolutional Neural Networks where it can analyse behavioural patterns and classify risk levels into 3 categories which are Low, Moderate, and High. The system utilise an explainable AI techniques by highlighting the factors that contribute to the assessment. According to the article, an experimental evaluation is achieving an accuracy of 89.40% which shows that the platform is user-friendly and accessible where parents and caregivers can perform the screening at home easily (Joshi et al., 2026).

Table 1: Comparison of existing systems

Features	M-CHAT-R	Manual BPPOKU	AutiScan	Proposed System
Autism screening questionnaire	Yes	No	Yes	Yes
Automated digital risk scoring	No	No	Yes	Yes
Digital Profile Management	No	No	No	Yes

Table 2: Issues or Problems with Existing Systems

Features	M-CHAT-R	Manual BPPOKU	AutiScan
Core technical issues	High false-positive rate where children keep getting positive result although they are not	Severe procedural delays waiting for medical sign-offs	AI prediction depends on model performance
Human element problem	Parents or caregivers misinterpreting or answering out of denial	Stigma surrounding the ‘disable’ label prevents parents signing up their children	Children may behave differently during video recording which can cause unreliable analysis
Main limitation	Requires follow-up assessment when indicated	Requires manual documentation and medical verification causing registration process becomes more time consuming	Screening results are preliminary and should not be treated as a formal diagnosis

3.0 Proposed System

The recommended solution is known as the Autism Early Screening Support System, which is an advanced two-fold application Web and Mobile technology system. The system combines standard medical screening procedures with daily management systems to create an efficient technological platform for the use of parents and NGOs.

3.1 Core System Infrastructure and Security

The foundation of the system lies in data security and privacy.

3.1.1 Authentication and Access Controls

The system uses safe methods for logging into and out of the system, which ensures that confidential data is protected by not allowing access to people who are not authorized to see the data.

3.1.2 Registration and Account Management

A separate module of the system is used to manage users' accounts. This module allows new users to register for an account by providing the required information such as their name, email address and password. After successful registration, users can log in to the system using their registered email address and password. The system verifies their login credentials to ensure that only authorised users can access their accounts. Users can also view and update their account information when necessary. In addition, the system provides secure account management by protecting users' personal information and password. If a user forgets their password, the system may provide a password recovery function to help them recover the access to their account.

3.1.3 Data Security and Privacy

Considering the confidentiality of data in questions, data security is a critical feature of the core infrastructure of the proposed solution, as it will serve as a reliable environment for the formative years intervention program.

3.2 Standardized Screening & Profile Management

This module digitalizes the old screening processes manually performed to guarantee accuracy and fast intervention.

3.2.1 Digital Profile Management (BPPOKU Pindaan 2019)

The program will digitalize Bahagian A (Biodata) from the BPPOKU (Pindaan 2019) form. This process will include the following important pieces of information:

- Personal Details

  Name, MyKad / MyKid identification, and contact details.

- Education & Professional Experience

  Education level and parents/guardians' employment.

- Details of Next of Kin

  The system collects detailed information about relatives close to the person for further communication in case of necessity.

3.2.2 M-CHAT-R Screening Integration

This feature includes a test with 20 questions performed according to the M-CHAT-R standard. The users provide "Yes" or "No" answers regarding children's behavior, such as do they react to a gesture or respond when called?

3.2.3 Automated Scoring Algorithm of Risk Level

The system will be able to provide risk levels automatically, making sure that each child undergoes an evaluation process that is not only consistent but also standardized and objective in nature. For those children with results falling within the Low Risk category, which would include any score of 0-2, there would not be a need to pursue any additional actions unless there are any future worries. Those with scores ranging from 3-7 are considered to have Moderate Risk, automatically prompting notification about the requirement of taking some actions to further examine their behavior. In instances where the scores range from 8-20, the result will be classified as a High Risk automatically, skipping other steps to indicate the urgent requirement of referral of the client for professional diagnostic testing.

3.3 Support, Analytics and Specialized Management

In addition to the diagnosis, this system features modules which enable monitoring and assistance on a continuous basis.

3.3.1 Automated Notification System

To resolve the issue of "delayed referrals" in the conventional approach, this module helps generate automated alerts regarding upcoming screenings or follow-up consultations.

3.3.2 Reporting and Analytics

Data analytics provide an opportunity to understand how the development of a child evolves throughout time, as visualized by parents or NGOs.

3.3.3 Behavior Monitoring

This module monitors the emotional state and mood of the child, enabling caregivers to identify any possible triggers for certain behaviors. These are some of the functions of this module:

- Behavior Log Processor

  A real-time function that enables the caregiver to log emotional status, particular behavioral instances, and the corresponding environmental antecedents.

- Trigger Analyzer

  A pattern-matching algorithm that scans historical logs to identify correlations between environmental factors that repeat themselves and behavioral disruptions that occur again.

- Mood Trend Evaluator

  A computation function that calculates and charts baseline emotional movements over specified timelines, providing parents and NGOs with empirical evidence of structural improvement.

3.3.4 Appointment Scheduling and Referral Management

This module allows parents to book appointments directly with specialist and referral centres, as well as with education centres, through the system. When a child's behavioural log or screening result shows a concerning trend, parents can make an appointment without needing to rely on a phone call, resolving the ambiguity and delay parents currently face once the limited monthly clinical sessions come to an end. The module checks real-time availability of the specialist or

referral centre, records the appointment slot, and automatically issues a referral letter and appointment reminder to the relevant party. This directly replaces the current appointment method and the phone call and manual diary check process, which currently takes 1 to 3 working days and produces no automatic reminder.

3.3.5 Educational Placement and Special Education Management

Through the 3.3.4 module, parents are allowed to search, filter, and view the physical locations of specialized education and early intervention centres within their vicinity. When a child's screening result or behavioral logs indicate a need for early learning support, parents can quickly locate nearby facilities, view available services, and access contact information without any manual search delays. This bridges the gap between clinical diagnosis and educational placement by providing a centralized directory of specialized schools.

3.3.6 Community Groups and Peer Support

An interactive communication hub provides a dedicated discussion platform where parents and caregivers can connect, share experiences, and offer mutual peer support. Users can join localized or topic-specific forums to exchange and navigate educational systems. By centralizing communication within a secure environment, system users can reduce the isolation felt by caregivers and eliminate the scattered nature of relying on external social media channels.

4.0 Software Process Model

4.1 Selected Software Process Model

The software process model chosen for the project is the Incremental Development Model. In this model, the system's main increments have been identified in advance, but detailed scope and order of increments can be adjusted based on customer feedback and priorities.. Each increment added incorporates functionalities that meet customer's expectations. As such, the model allows the project to progress smoothly due to the flexibility in developmental phases.

4.2 Justification on Selected Software Process Model

The Incremental Development Model have been chosen based on the following reasons :

- Cost of Accommodating Changing Requirements
  Cost is reduced as the amount of analysis and documentation that has to be redone as requirements are changed are significantly less.

- Modular System with Clear Dependencies
  The project has been separated into modules with dependencies explicitly known and mapped out. Modules such as Authentication, BPPOKU Profile, and M-CHAT-R Screening must exist before modules including Notifications, Appointments, and Reports can work, so each module can be delivered in a natural order.

- Ease of Acquiring Continuous Customer Feedback
  The model makes it easier to obtain customer feedback on the work that has been done, as every increment is reviewed together with parents, NGO administrators and healthcare staff. The reviews are used to correct usability problems before the next increment begins.

- Rapid Delivery and Deployment
  Delivery and deployment of useful software are released at a faster pace as the large systems are broken down into smaller, standalone function pieces or increments that are built, tested, and released one by one.

- Low Risk for Sensitive functions
  Modules that handle private information data and correctness of risk scoring carries the highest risk towards the system. With the given approach, modules are built and tested first to prevent errors in later features that can endanger the core.

- Suitability with Team and Schedule

With a team of five and a project timeline of 15 weeks, members can work in parallel to build the system. As the approach includes prioritized feature releases, core functionalities and a usable system can exist even if the schedule is tight.

4.3 Detailed Software Development Life Cycle (SDLC) Activities of Each Phases

The Software Development Life Cycle (SDLC) is a phased approach to solving business problems. SDLC is developed through the use of a specific cycle of analyst and user activities, with each phase having its own unique user activities. SDLC of the Incremental Development Model includes 7 phases, which are used to properly plan the project schedule and requirements.

4.3.1 Phase 1: Initiation and Planning

- ➢ Define the project scope, objectives and stakeholders.
- ➢ Carry out technical, operational, and economic feasibility studies, and identify the risks along with mitigation actions.
- ➢ Group the modules into increments by dependency and value, and confirm the overall architecture.
- ➢ Set up team organisation and tools, such as GitHub repository, NexScholar task board, Whatsapp communication group, and Figma.
- ➢ Prepare the project plan, schedule, and all relevant plannings.

4.3.2 Phase 2: Requirements Analysis

- ➢ Obtain requirements through questionnaires and observation with parents, NGO administrators and healthcare staff, and review the current manual workflow, which is the M-CHAT-R checklist and the BPPOKU form.
- ➢ Analyze the document functional requirements and non-functional requirements.
- ➢ Determine key user interface designs tailored to parents, caregivers, and NGO administrators to ensure the project remains easily accessible.

4.3.3 Phase 3: System Design

- ➢ Refine the architecture and separate increments into modules using structure charts and physical DFDs.
- ➢ Design the database with integrity constraints and protection of sensitive fields.
- ➢ Design the user interface by refining Figma wireframes for the parents/ caregivers and NGO portal by applying HCI principles and reviewing them with users.
- ➢ Design algorithms and interfaces, such as M-CHAT-R scoring and risk classification, repeated pattern detection, and mood trend calculation, and links to the notification service.

4.3.4 Phase 4: Implementation

- ➢ Develop the database, back-end services and business logic, the web portal pages according to the approved design.
- ➢ Use Git feature branches, pull request and peer code reviews, and integrate the new code with previous increments.

- ➢ Keep technical documentation up to date, track tasks on the dashboard and hold short weekly progress meetings.
- ➢ Write codes together and fix errors found during reviews.

4.3.5 Phase 5: Testing

- ➢ Perform acceptance testing with the implemented code.
- ➢ Carry out functional tests and non-functional tests.
- ➢ Log, fix and test code again to ensure the system has no defects.

4.3.6 Phase 6: Deployment and Increment

- ➢ Build the software, back up data, and deploy to the staging and production environment.
- ➢ Provide user support services such as Help Center, quick guides and short training for NGO staff.
- ➢ Perform an increment review to record lessons learned, update the requirement backlog, and confirm scope of the next increment.

4.3.7 Phase 7: Final Integration and Maintenance

- ➢ Perform full system integration, security and performance check.
- ➢ Conduct final documentation and project sign-off.

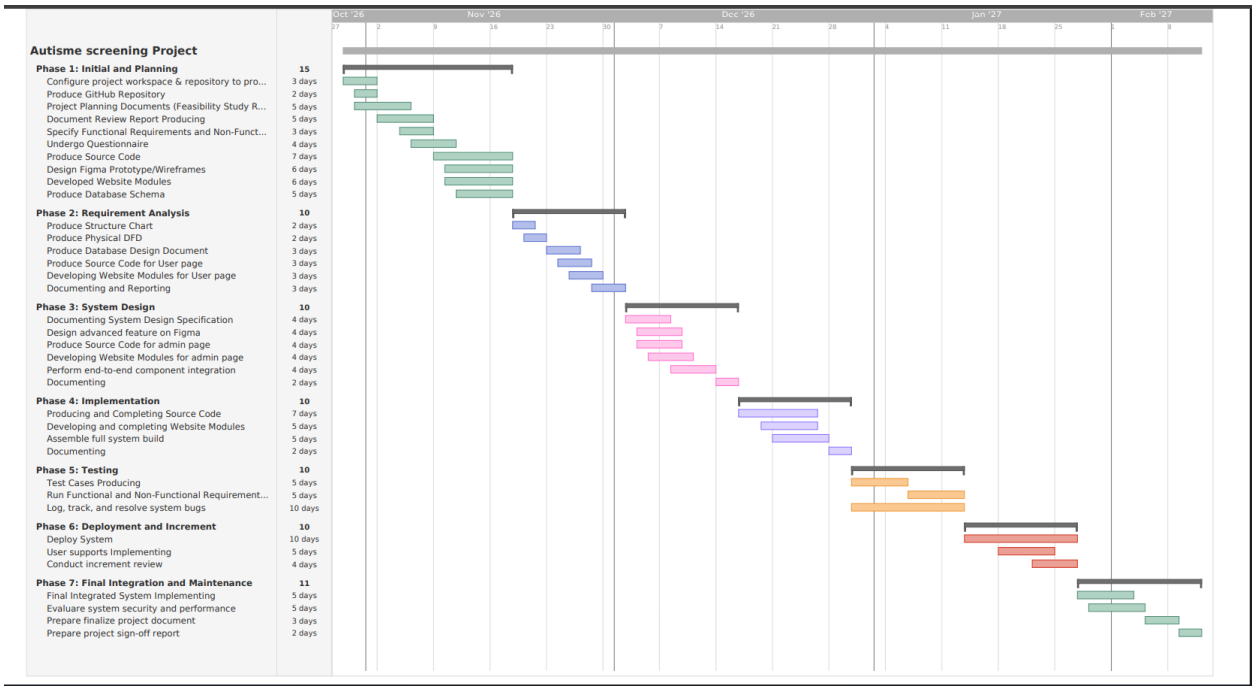
4.4 Duration, Person-in-Charge (PIC), and Deliverables for Each SDLC Phase

Phase	**Duration**	**Person-In-Charge (as leader of the sprint)**	**Sprint Goal**	**Deliverables**
Phase 1: Initial and Planning	Week 1-2 (2week)	Balqis Batrisya binti Jalaluddin	Project setup, initial requirements, developing first module(user authentication and base environment)	-Team Setup Documentation (GitHub Repository, NexScholar Board) -Project Planning Documents (Feasibility Study Report, Project Proposal, Project Schedule) -Document Review Report -Questionnaire Report -Functional Requirements and Non-Functional Requirements Reports -Figma Prototype/Wireframes

				-GitHub Repository -Database Schema -Source Code -Developed Website Modules
Phase 2: Requirement Analysis	Week 3-4 (2 weeks)	Nasaaie bin Noriskamar	Designing, implementing and integrating the first main functional module(user module)	-Structure Chart -Physical DFD -Database Design Document -Source Code -Developed Website Modules -Integrated System Components -Technical Documentation
Phase 3: System Design	Week 5-6 (2 weeks)	Yong See En	Expanding system modules and processing logic	-System Design Specification -Figma Prototype/Wireframes -Source Code -Developed Website Modules -Integrated System Components -Technical Documentation
Phase 4: Implementation	Week 7-8 (2 weeks)	Balqis Batrisya binti Jalaluddin	Completing remaining website modules and report documentation	-Source Code -Developed Website Modules -Integrated System Components -Technical Documentation
Phase 5: Testing	Week 9-10 (2 week)	Nasaaie bin Noriskamar	Testing and reporting	-Test Cases -Bug Report -Functional Testing Report and Non-Functional Testing Report
Phase 6: Deployment and Increment	Week 11-12 (2 week)	Han Tian Chou	Launching system and preparing end-user supports	-Deployed System -User Supports including user manual and quick user guide implementing -Increment Review Report

Phase 7: Final Integration and Maintenance	Week 13-14 (2 week)	1)Qistina Batrisyia binti Noor Mohd Azlan	Final production verification, security assessment and project sign-off	-Final Integrated System -Security and Performance Report -Final Documentation -Project Sign-off Report

5.0 Project Schedule



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7.0 Appendices

M-CHAT-R™

Please answer these questions about your child. Keep in mind how your child usually behaves. If you have seen your child do the behavior a few times, but he or she does not usually do it, then please answer **no**. Please circle **yes** or **no** for every question.

1. If you point at something across the room, does your child look at it? (**FOR EXAMPLE**, if you point at a toy or an animal, does your child look at the toy or animal?)	Yes	No
2. Have you ever wondered if your child might be deaf?	Yes	No
3. Does your child play pretend or make-believe? (**FOR EXAMPLE**, pretend to drink from an empty cup, pretend to talk on a phone, or pretend to feed a doll or stuffed animal?)	Yes	No
4. Does your child like climbing on things? (**FOR EXAMPLE**, furniture, playground equipment, or stairs)	Yes	No
5. Does your child make unusual finger movements near his or her eyes? (**FOR EXAMPLE**, does your child wiggle his or her fingers close to his or her eyes?)	Yes	No
6. Does your child point with one finger to ask for something or to get help? (**FOR EXAMPLE**, pointing to a snack or toy that is out of reach)	Yes	No
7. Does your child point with one finger to show you something interesting? (**FOR EXAMPLE**, pointing to an airplane in the sky or a big truck in the road)	Yes	No
8. Is your child interested in other children? (**FOR EXAMPLE**, does your child watch other children, smile at them, or go to them?)	Yes	No
9. Does your child show you things by bringing them to you or holding them up for you to see – not to get help, but just to share? (**FOR EXAMPLE**, showing you a flower, a stuffed animal, or a toy truck)	Yes	No
10. Does your child respond when you call his or her name? (**FOR EXAMPLE**, does he or she look up, talk or babble, or stop what he or she is doing when you call his or her name?)	Yes	No
11. When you smile at your child, does he or she smile back at you?	Yes	No
12. Does your child get upset by everyday noises? (**FOR EXAMPLE**, does your child scream or cry to noise such as a vacuum cleaner or loud music?)	Yes	No
13. Does your child walk?	Yes	No
14. Does your child look you in the eye when you are talking to him or her, playing with him or her, or dressing him or her?	Yes	No
15. Does your child try to copy what you do? (**FOR EXAMPLE**, wave bye-bye, clap, or make a funny noise when you do)	Yes	No
16. If you turn your head to look at something, does your child look around to see what you are looking at?	Yes	No
17. Does your child try to get you to watch him or her? (**FOR EXAMPLE**, does your child look at you for praise, or say "look" or "watch me"?)	Yes	No
18. Does your child understand when you tell him or her to do something? (**FOR EXAMPLE**, if you don't point, can your child understand "put the book on the chair" or "bring me the blanket"?)	Yes	No
19. If something new happens, does your child look at your face to see how you feel about it? (**FOR EXAMPLE**, if he or she hears a strange or funny noise, or sees a new toy, will he or she look at your face?)	Yes	No
20. Does your child like movement activities? (**FOR EXAMPLE**, being swung or bounced on your knee)	Yes	No

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Figure 1: M-CHAT-R questionnaire
https://marf.org.my/mchatrf/

2. **How to apply**

There are two ways to register:

- **Manually**

Submission of BPPOKU documents (Amendment 2/2019) at the JKM counter for the registration process

- **Online via MyDaftar OKU**

Visit MyDaftarOKU >

Figure 2: BPPOKU registration ways
https://www.malaysia.gov.my/en/categories/personal-identification/oku-card-application/oku-registration

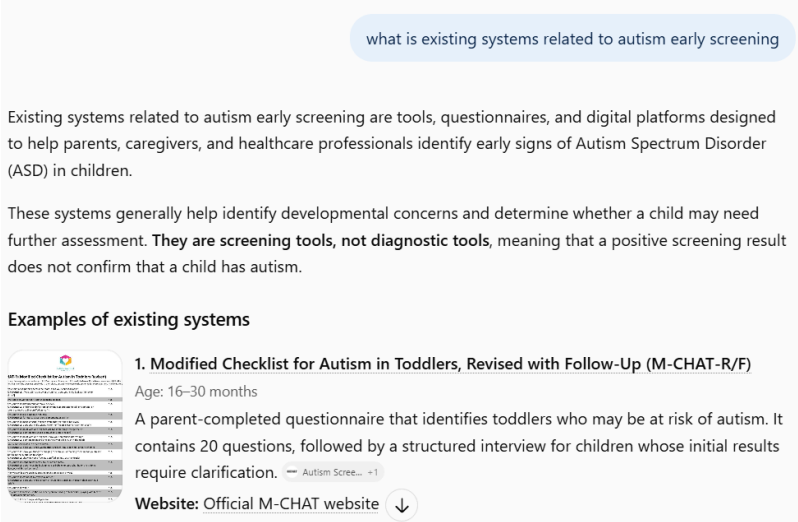


Figure 3: AI prompt on ChatGPT
https://chatgpt.com/share/6add148c-ea6c-83ec-a2cb-9da2b61a3ea5

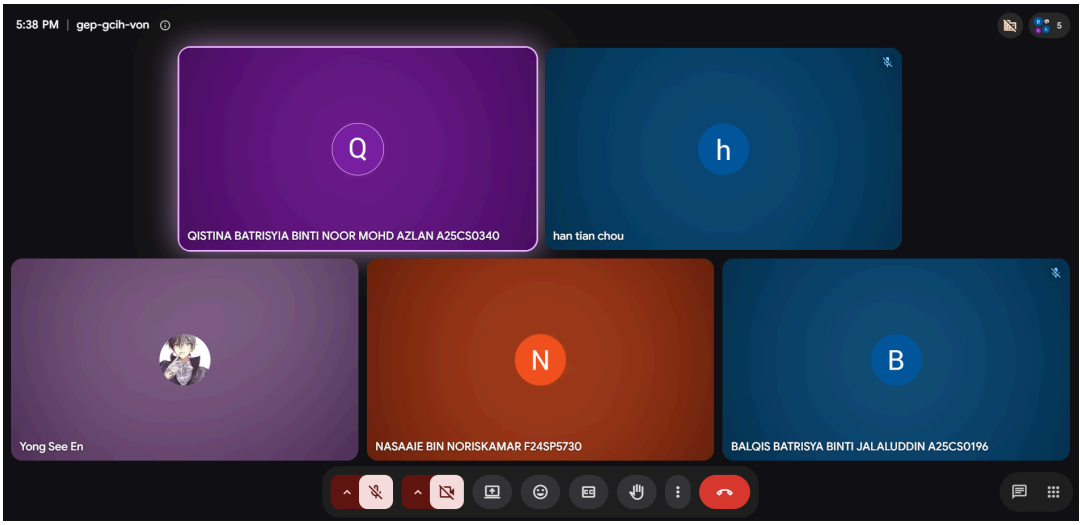


Figure 4: Project Proposal Discussion via Google Meet